

00:00 → 02:00

Hi everyone!

Today I will be talking about a movie that critics claim put the Odia film industry back on the world map.

The name of the movie is DAMaN, which stands for Durgama Anchalare Malaria Nirakarna, which means Malaria Eradication in Remote Areas.

I will be following a very linear thematic analysis of this movie, primarily exploring the deeper question:

“What actually determines whether healthcare works or fails?”

Since almost no one has seen the movie, let me start with a brief summary.

The film is set in 2015 and follows Siddharth, a young doctor who is required to serve in a remote tribal area of Odisha due to a government bond.

He is posted to a primary health center in a region with around 150 villages, where there are almost no basic facilities and limited access to healthcare.

Initially, he struggles to adapt and even tries to leave.

However, he soon realizes that many villagers are suffering from malaria but do not seek medical treatment. Instead, they rely on traditional beliefs, often treating illness as something supernatural.

He begins traveling across villages, conducting tests, distributing medicines, and trying to spread awareness.

Despite early efforts, many interventions fail. For example, mosquito nets provided by the government are not used properly, and malaria continues to spread.

Over time, he works with local people and authorities to develop a more structured campaign.

After several years of sustained effort, malaria cases are significantly reduced, and he eventually leaves the village.

The story, however, is less about his journey and more about the challenges of delivering healthcare in such conditions, as I mentioned previously. Now we will follow the flow of the film and explore the question more deeply.

02:00 → 03:30

At the beginning of the film, Siddharth does not want to stay in the village. But this reluctance is not simply personal. It reflects a deeper structural problem.

He has been trained in an urban system that provides:

- infrastructure
- support
- and predictability

And suddenly, he is placed in an environment where none of these exist.

There's nothing here... no proper hospital, no electricity... how is anyone supposed to work like this?

This moment is important because it shows that the issue is not unwillingness to serve. It is the conditions under which service is expected.

This idea is not limited to this film.

We see a similar pattern in Swades, where the protagonist returns from an urban, global environment and initially struggles to adapt to rural life. The discomfort there is also not hostility, but unfamiliarity with a completely different system.

And beyond cinema, this has been discussed in development economics as well.

For example, economists like Abhijit Banerjee argue that policies which rely only on compulsion, like the government bonds in the movie, without improving working conditions, often lead to compliance without real engagement.

So what we see here is part of a larger pattern:

Systems try to solve shortages by forcing presence, but they do not create the conditions needed for meaningful work.

But even when a doctor is present, another challenge emerges—one not of infrastructure, but of communication.

03:30 → 05:45

At first, the problem in the film looks simple. Villagers believe in supernatural causes of illness, while the doctor relies on scientific explanations. When we assume that one side is wrong and the other is right, we miss the nuance that they are operating within completely different systems of understanding.

You think this is a spirit? This is an illness. Why won't you understand?

This moment is important because it shows that being correct is not enough. The doctor has the right diagnosis, but he does not yet have the ability to communicate it in a way that is accepted.

This idea is also discussed in medical anthropology.

Arthur Kleinman makes a distinction between disease and illness. Disease is the biological condition, but illness is how people experience and interpret that condition within their culture. So while malaria is a disease for the doctor, for the villagers, it is understood through their own system of meaning.

We see a similar pattern in Pad Man. The problem there is not the absence of a solution, but the inability to integrate it into existing social norms. The product exists, but acceptance comes only after communication is reframed.

This is why local actors become so important in the film. People like the pharmacist are not just assistants. They act as bridges between formal medicine and the community. They help translate not just language, but meaning.

So the real issue here is not superstition. It is disconnection. Medicine does not fail because it is incorrect—it fails because it cannot enter the social world of the people it is meant to serve.

05:45 → 07:30

But healthcare is not just about doctors or medicine. It is about whether people can physically access care at all.

In the region shown in the film, villages are:

- scattered across forests
- separated by hills
- and divided by rivers

So reaching a hospital is not simple. It is often a long and difficult journey. At one point, when a patient needs urgent care:

We have to carry her... there is no other way.

This moment is critical because it shows that treatment does not begin at diagnosis—it begins with movement.

If a patient cannot reach a hospital, then for them, the hospital effectively does not exist. This is not a medical failure. It is an infrastructural failure. The doctor is present, the patient needs care, but the system cannot connect the two.

This reflects what public health research consistently shows. One of the biggest barriers to healthcare is not treatment—it is access. Distance, transport, and geography often determine whether care is even possible.

We can think of healthcare in three layers:

- availability
- accessibility

- and usability

And what the film clearly shows is that in rural areas, even if you fix availability by posting doctors, you still have to solve accessibility and usability. Otherwise, the system will fail.

And when access is difficult, people adapt. They delay treatment or rely on alternatives that are closer and easier, even if they are less effective or completely superstitious.

07:30 → 09:30

Now even when access improves, another problem becomes visible.

And that is the gap between policy and practice.

In the film, mosquito nets are distributed to prevent malaria. On paper, this looks like a successful intervention—coverage increases, resources are delivered, and the problem appears to be addressed.

But in reality, something very different happens.

They are using the nets... but not for sleeping.

This moment is important because it reveals a limitation in how policy is often designed.

Policies tend to assume that once a solution is provided, its purpose is automatically understood and adopted. But in practice, people do not interact with resources in ideal or predefined ways.

Instead, they interpret and use them based on their immediate needs, habits, and environment.

So a mosquito net is not just seen as a medical tool—it becomes an object that can serve multiple purposes depending on context.

Giving is easy... making people use it is the real challenge.

This highlights a deeper issue.

The gap here is not just between policy and people, but between design and lived reality.

Public health interventions often operate at a distance, where success is measured in distribution numbers.

But real success depends on whether those interventions are actually integrated into everyday life.

This requires more than providing resources.

It requires:

- continuous engagement
- behavioral understanding

- and adaptation to local context

Without this, even effective solutions remain ineffective in practice.

So the failure here is not in the tool itself, but in assuming that delivery is the same as adoption.

Because in public health, outcomes are determined not by what is given—but by what becomes part of daily life.

09:30 → 12:00

Finally, even after all these challenges, there is one deeper issue that the film reveals—and that is the limits of individual effort.

At first, the doctor appears as a hero. He travels across villages, treats patients, and spreads awareness. But if we look closely, something more fundamental is happening.

He is not just doing his job.

He is compensating for the absence of a system.

He is:

- transporting patients
- explaining medical knowledge
- distributing resources
- and coordinating efforts

These are not individual responsibilities. These are functions that should be distributed across institutions, infrastructure, and trained networks.

At one point, the scale of the problem becomes clear:

There are so many villages... how can one person reach all of them?

This is not just a question of effort. It is a question of structural impossibility. Because no individual, no matter how committed, can sustain this level of responsibility over time.

And this becomes even clearer when it is said:

If we don't go, no one will.

This line reveals a deeper problem. The system works only when individuals overextend themselves, and that makes it fragile. Because the moment the individual steps back, the system collapses.

This is why healthcare systems research consistently emphasizes that good outcomes do not come from exceptional individuals, but from reliable and repeatable systems. Writers like Atul Gawande argue

that consistency, coordination, and structure are what make care effective—not isolated effort.

The real shift in the film happens when the model changes. When the work moves from one person to a structured campaign, responsibility is no longer concentrated—it becomes distributed. And that is what makes it sustainable.

As the idea is expressed:

It cannot depend on one person... it has to become a system.

So the key takeaway is this:

Individual effort can initiate change.

But only systems can sustain it.

12:00 → 14:00

So what does all of this tell us?

The film is not just about a doctor fighting malaria.

It is about how healthcare systems actually function.

Across all these themes, we see the same pattern.

Healthcare does not fail at the point of treatment.

It fails in the conditions that make treatment possible— in access, in communication, in policy design, and in system structure.

What initially appears to be a medical problem gradually reveals itself to be a systemic one.

Because even the most effective treatments remain limited if they cannot reach people, if they are not trusted, or if they are not used in practice.

Healthcare exists not where medicine is available, but where systems allow it to be accessed and used.

This reframes the problem entirely.

Malaria, in this story, is not just a disease.

It is an outcome shaped by infrastructure, by social understanding, and by the way policies are designed and implemented.

It is a reminder that public health is not a single intervention, but a network of conditions that must work together.

And perhaps most importantly, the film challenges how we think about solutions.

It shows that relying on individual effort, no matter how committed, is not enough.

Because effort can bridge gaps temporarily, but it cannot remove them.

Effort can start change, but only systems can sustain it.

So in the end, the story is not just about what one doctor achieved.

It is about what becomes possible when systems begin to work.